



HOT TOPICS IN Pediatric Nursing

Pediatric Pain Remains Undertreated

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Despite advances in pediatric pain management, pain in children is often underrecognized and undertreated. Compared with adults with the same diagnosis, children receive fewer analgesic doses, with younger children and infants suffering the most (Friedrichsdorf & Goubert, 2019). Hospitalized neonates are particularly vulnerable as they experience on average 7 to 17 painful procedures per day and in many cases no analgesia is provided (Anand et al., 1999; Roofthoof et al., 2014). Children with serious medical conditions and those undergoing surgical procedures are frequently exposed to painful procedures such as bone marrow aspi-

therapy. Evidence overwhelmingly points to a “bundle” of four modalities: topical anesthesia, comfort positioning, sucrose or breastfeeding for infants, and age-appropriate distraction for more common procedures such as blood draws, injections, or vaccinations. Nitrous gas analgesia and sedation may be necessary and appropriate for some children. Opioid use, though not without risk, is recommended for medium-to-severe pain due to tissue injury and must be monitored closely for side effects and adverse reactions in the acute care setting (Friedrichsdorf & Goubert, 2019). Although there have been reports of future opioid misuse following legitimate prescrip-

leagues (Czarnecki et al., 2019). Wrona and Czarnecki conclude with a call to pediatric nurses to make a difference by committing to using individualized and age-appropriate pain assessment with valid and reliable scales, treating pain with multimodal approaches, involving parents in pain management during procedures, educating children and parents about pain management choice, and advocating for optimal pain management with the interprofessional team. ✚

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Nurses must make sure pain relief measures are used for all pediatric patients when needed.

rations, lumbar punctures, venipunctures, and dressing changes. Even healthy children are exposed to painful procedures on a frequent basis, with vaccinations the most common (Friedrichsdorf & Goubert).

Friedrichsdorf and Goubert (2019, p. 1) review best evidence and practice for the treatment of pediatric pain and conclude that “failure to implement evidence-based pain prevention and treatment for children in medical facilities is now considered inadmissible and poor standard of care.” For hospitalized children, multimodal analgesia is the gold standard of care and includes pharmacology, regional anesthesia, rehabilitation, psychology, spirituality, and integrative modalities. For children with chronic pain such as migraines or tension headaches, abdominal pain, and musculoskeletal pains, an interdisciplinary approach such as physical therapy, psychological approaches, integrative mind-body therapies, and a normalization of life style is the best

tion, the vast majority of opioids misused by children are prescribed or obtained from family or friends (Allen et al., 2017).

Nurses are critical to the prevention, assessment, and treatment of pediatric pain. Wrona and Czarnecki (2021) present a summary of the critical role of nurses, highlighting perceived barriers to effective pain management, valid and age-appropriate pain assessment scales, and affirmation of an individualized, multimodal, and interprofessional approach. Pharmacologic and non-pharmacologic pain management approaches are summarized for children with acute and chronic pain. The most frequently identified barriers to effective pediatric pain management by nurses included: inadequate provider orders, insufficient time to provide pain management prior to a procedure, insufficient premedication orders, and a low priority given to pain management by medical col-

References

- Allen, J. D., Casavant, M. J., Spiller, H. A., Chounthirath, T., Hodges, N. L., & Smith, G. A. (2017). Prescription opioid exposures among children and adolescents in the United States: 2000-2015. *Pediatrics*, 139(4), e20163382. <https://doi.org/10.1542/peds.2016-3382>
- Anand, K. J., Barton, B. A., McIntosh, N., Lagercrantz, H., Pelusa, E., Young, T. E., & Vasa, R. (1999). Analgesia and sedation in preterm neonates who require ventilatory support: Results from the NOPAIN trial. Neonatal outcome and prolonged analgesia in neonates. *Archives of Pediatrics and Adolescent Medicine*, 153(4), 331-338. <https://doi.org/10.1001/archpedi.153.4.331>
- Czarnecki, M. L., Guastello, A., Turner, H. N., Wrona, S. K., & Hainsworth, K. R. (2019). Barriers to pediatric pain management: A brief report of results from a multisite study. *Pain Management Nursing*, 20(4), 305-308. <https://doi.org/10.1016/j.pmn.2019.01.008>
- Friedrichsdorf, S. J., & Goubert, L. (2019). Pediatric pain treatment and prevention for hospitalized children. *Pain Reports*, 5(1), e804. <https://doi.org/10.1097/PR9.0000000000000804>
- Roofthoof, D. W. E., Simons, S. H. P., Anand, K. J. S., Tibboel, D., & van Dijk, M. (2014). Eight years later, are we still hurting newborn infants? *Neonatology*, 105(3), 218-226. <https://doi.org/10.1159/000357207>
- Wrona, S., & Czarnecki, M. L. (2021). Pediatric pain management. *American Nurse Journal*, 16(3), 6-12.